

PATIENT: BLACKWELL, THEODORE R DOB: 09/14/1957 MRN: E4085221

ADMISSION DATE: 04/26/2026 DISCHARGE DATE: 04/30/2026

ATTENDING: Robert Chen, MD (Hospital Medicine)

ADMISSION DIAGNOSIS

Acute decompensated heart failure.

DISCHARGE DIAGNOSES

1. Acute on chronic systolic heart failure (HFrEF, LVEF 35-40%), volume overload, improved.
2. Atrial fibrillation with controlled ventricular response, on apixaban.
3. Stage III sigmoid colon adenocarcinoma, on adjuvant FOLFOX (held during admission, oncology following).
4. Chronic kidney disease stage 3 with acute kidney injury on admission, resolved to baseline.
5. Type 2 diabetes mellitus.

HOSPITAL COURSE

Mr. Blackwell presented to the Emergency Department with three days of progressive dyspnea, orthopnea, and 4 kg weight gain after cycle 2 of FOLFOX. BNP 1890, chest x-ray with pulmonary vascular congestion and small bilateral pleural effusions. He was diuresed with IV furosemide with 5.2 L net negative over the stay. Creatinine peaked at 1.9 and returned to baseline 1.5. Rate remained controlled in atrial fibrillation; apixaban continued. Echocardiogram showed LVEF 35-40%. Empagliflozin added. Chemotherapy held during admission and resumed per outpatient oncology.

DISCHARGE MEDICATIONS

Apixaban 5 mg BID; metoprolol succinate 100 mg daily; sacubitril-valsartan 24-26 mg BID (uptitration per cardiology); spironolactone 25 mg daily; empagliflozin 10 mg daily (NEW); furosemide 40 mg daily; metformin 1000 mg BID; ondansetron PRN.

FOLLOW-UP / DISPOSITION

Discharged home in improved condition. Cardiology (Dr. Deshpande) in 2 weeks; oncology (Dr. Oyelaran) in 1 week; labs (BMP) in 5 days. Strict return precautions reviewed, including neutropenic fever precautions while on chemotherapy.

Electronically signed: Robert Chen, MD – Hospital Medicine